**جوستاف روسي انترناشيونال - مصر (EPE) ***

# طلب علاج أول مرة / متردد

**إسم المريض رباعياً :** جيهان حسين ابراهيم خليل
**المحافظة :** الجيزه **الحي / المدينة :** الجيزه **العنوان :** فرحات
**رقم البطاقة :** 27511192301382 **رقم التليفون :** 01061496411 **تاريخ تسجيل الإستمارة :** 11-10-2026
**العمر(تاريخ الميلاد) :** 19-11-1975 **المهنة :** ربه منزل

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** علاج مبادرة سرطان الثدي مزيد من الأبحاث والفحوص الإشعاعية [7954] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنه و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

Name: جيهان حسين إبراهيم خليل
National ID: 27511192301382

## تقرير طبي

السيد

تحية طيبة وبعد,,

| | |
|---|---|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم: | رقم طبي: |

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

| | |
|---|---|
| تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd | تاريخ وساعة الخروج : PM 01:17 05-07-2026 |

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further laboratory investigations and radiological studies.

| | |
|---|---|
| التاريخ : 05-07-2026 | الوقت : -- --:-- |
| | الطبيب المعالج : د/شيماء لاشين |

(١)



٢٧٥١١١٩٢٣٠١٣٨٢ ٢٠١٩/١٢

ربة منزل

أنثى مسلمة متزوجة

محمد عبدالواحد محمد محمد

البطاقة سارية حتى ٢٠٢٦/١٢/٠٩

١





جمهورية مصر العربية

بطاقة تحقيق الشخصية

جيهان

حسين ابراهيم خليل

٢٩ش قرحات

الجيزه - الجيزه

٢٧٥ ١١ ١٩ ٢٣ ٠١٣ ٨٢

IG6219514

٤٧

٠١٠٦١٤٩٦٤١١

فتكم ملف عدد جزء

امثال سيد

٢٤٥٢

٥ ٤

ERM036725051

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

08/04/2026

# MDT FORM

| | | | |
|---|---|---|---|
| **Name:** | Jihan Huseen Ibrahem . | **MRN:** | 1384 |
| **Age:** | 51 Y | **ID:** | 27511192301382 |
| **Menopausal Sta** | | **Phone:*** | 01100264411 |
| **NRS SCORE:** | | **Marital Status:** | |
| **BMI:** | | **Family History:** | |
| **Gender** | | | |
| **CO Morbidity:** | | **Reason Of MDT:** | Metastatic |
| **Surgical History:*** | BCS | **ECOG Performance Status** | ○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 |

## Clinical Data:

**BX:** IDC

**ER:** 0/8 **PR:** 6/8 **HER2:** 3+ **KI67:**

**History:**

female Pt 50 yrs pre HTN +ve FH her mother thyroid cancer >> RT BC T2 N0 >>> ER 0/8 PR 6/8 her2+3 IDC >>> upfront BCS ( 12-2024) + SLNB >>> adj Cth AC *4 TXT *4 >>PORT >> Pt didnot receive anti her2 >> on tam since 6-2024 till now O/E>> Rt inflammatory breast with rigidity BSM 5-2026 >>> Lt axillary LNs about 1.2*0.8 cm , extensive infiltrative involving rt breast BSM 18-6-2026 >> Rt extensive inflammatory carcinomatosis of rt breast

O/E: Rt breast locally advanced disease + skin nodules on abd wall

Complaining from cough and dyspea - impending visceral crisis

**Sonomammography:**

**Biomarker/Molecular**

Pending cytology from effusion

**Mets Workup:**

CT CAP: new LT Axillary LN + Rt sided marked pleural effusion + Newly developed osteolytic bony lesions.

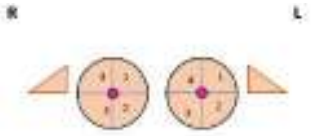
Pending Bone scan

**Recommendations:**

Navelbine + Herceptin + Zometa

**MDT Outcome & Plan**

Navelbine + Herceptin + Zometa




TNM

| Right Side | | | | Left Side | | | |
|---|---|---|---|---|---|---|---|
| Q1 | Q2 | Q3 | Q4 | Q1 | Q2 | Q3 | Q4 |
| | | | | | | | |

**Clinical Staging:**

**Pathological Staging:**

## SIGNATURE

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY |
|---|---|---|---|
| د خالد عبد العزيز | د محمد نبراوي | | |

MDT Form
[[DOCTYPE:MDT_FORM]]

| | |
|---|---|
| **Name:** جيهان حسين ابراهيم | |
| **MR. :** 1384 | |

Detailed Medical Report

**السيد**

**تحية طيبة وبعد,,**

| | | | |
|---|---|---|---|
| **إيماء للطلب المقدم من** | | **بشأن تقرير طبي للمريض** | |
| **بقسم:** | | **رقم طبي:** | |

**ننشرف بأن نوضح فيما يلي تقريرا عن حالته**

**تاريخ وساعة الدخول :** **تاريخ وساعة الخروج :**

**وبعد الكشف والفحوصات والمتابعة وجد انه**

**The patient is a 50-year-old individual with a past medical history of hypertension and a positive family history of thyroid cancer in the mother. The clinical course began with a right-sided breast cancer classified as T2 N0, for which the patient underwent upfront breast-conserving surgery in December 2024 with sentinel lymph node biopsy, followed by adjuvant cyclophosphamide-based chemotherapy (four cycles) and radiotherapy. Anti-HER2 therapy was not administered at that time, and tamoxifen was started in June 2024 and continued to the present. On examination in May 2026, the patient presented with a right inflammatory breast exhibiting rigidity, skin nodules on the abdominal wall, and left axillary lymph nodes measuring approximately 1.2 × 0.8 cm, with extensive infiltrative disease of the right breast noted on June 18 2026. The patient reports cough and dyspnea, suggestive of an impending visceral crisis.**

**Histopathology confirmed invasive ductal carcinoma (IDC) with an estrogen-receptor score of 0/8, progesterone-receptor score of 6/8, and HER2 3+ positivity. Imaging demonstrated a right-sided marked pleural effusion, new left axillary lymphadenopathy, and newly developed osteolytic bone lesions on CT of the chest, abdomen, and pelvis; a bone scan is pending. These findings are consistent with locally advanced disease with cutaneous involvement and distant metastases.**

**The referring team recommended treatment with Navelbine, Herceptin, and Zometa. The multidisciplinary team concurred and the final plan is to proceed with Navelbine, Herceptin, and Zometa.**

**تحريرا في ,**

**التاريخ :** 23-09-2026 **الوقت :**

**الطبيب المعالج :**

**يعتمد ,**

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / جيهان حسين ابراهيم خليل | Request No. | : 13005914 |
| Patient No. | : 27511192301382 | Request Date | : 11/09/2023 |
| Path No. | : SR-1040/23 | Verification Date | : 28/09/2023 |
| Sex | : Female | Financial Category | : اورام دار السلام - اقتصادى |
| Age | : 47 Y | Department | : Out Patient |
| Referreing Doc : | | Registered by | : .marwa mohamed youss |

## Pathology Report

Clinical data
Right breast mass

Nature of specimen
Revision of slides
Gross examination
Two blocks ( 200779) (203291 + 6 slides.

Microscopic examination
* Histological type: Invasive duct carcinoma
* Grade ( differentiation +nuclear pleomorphism+mitosis ): (3+3+2) Grade III
* DCIS: No
* Angioinvasion: Could not be detected within the limits of biopsy received
IPT :
ER:No nuclear staining in tumor cells(0/8)
PR:strong nuclear staining in 40% of tumor cells (6/8)
Her2: Strong complete memrane staining in >10 of tumor cell , score( +3)
*Cytology&cell block: positive for malignant cells
Diagnosis
Right breast mass ; slide revision of tru cut biopsy specimen :
Invasive duct carcinoma, Grade III.
IPT :
ER: Negative (0/8)
PR:Positive(6/8)
Her2: Positive( +3)
Cytology&cell block: positive for malignant cells
Diagnosis

Signature

Verified by :
Dr.Heba Hasan



Room No. (4) - First Floor-Building No. (9), Nile Corniche - Sfier El Qobrasi- Cairo, Egypt

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / جيهان حسين ابراهيم خليل | Request No. | : 13005914 |
| Patient No. | : 27511192301382 | Request Date | : 11/09/2023 |
| Path No. | : SR-1040/23 | Verification Date | : 28/09/2023 |
| Sex | : Female | Financial Category | : اورام دار السلام - اقتصادى |
| Age | : 47 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa mohamed youss |

## Pathology Report

Prof. Dr. Amal Hareedy
Dr.Heba Mostafa

Verified by :
Dr.Heba Hasan



Room No. 140 - First Floor-Building No. 267 - Nile Corniche - Misr El Qadema- Cairo, Egypt

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / جيهان حسين ابراهيم خليل | Request No. | : 13008872 |
| Patient No. | : 27511192301382 | Request Date | : 05/12/2023 |
| Path No. | : 4324/23 | Verification Date | : 25/12/2023 |
| Sex | : Female | Financial Category | : اورام دار السلام - اقتصادى |
| Age | : 48 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa mohamed youss |

## Pathology Report

Clinical Data: Right breast cancer, Up-Front.
Nature Specimen: Right breast conservative surgery , sentinal lymph nodes and axillary clearance.

Gross Examination:
* Procedure: Right breast conservative surgery
Fibrofatty tissue piece measured 11.5x6x3 cm with covering skin ellipse measured 10x3 cm.
* Tumor extent: A wired greyish white firm tumor .
* Size: 1.8x1x1 cm.
* Distance from margins: 0.3 cm away from inferior margin ( least margin ).
* Rest of the breast showed greyish white areas with dilated ducts.
* Skin involvement: Unremarkable grossly
* Sentinal lymph nodes: Fibrofatty tissue piece measuring 3.5x2.5x1 cm entangling 10 lymph nodes.
* Axillary clearance : Fibrofatty tissue measured 8x5x2 cm entangling 20 lymph nodes .

Microscopic Examination:
* Histological type: Ivasive duct carcinoma .
* Grade (differentiation+ nuclear pleomorphism+ mitosis):(3+2+1) Garde II.
* Ductal carcinoma in situ: Yes ( high grade comedo )
* Resection margins: Free margins .
* Microcalcifications: Yes.
* Additional pathologic findings: Surrounding breast showed fibroadenosis .
* Lymph node involvement: Positive lymph nodes (16/30)

* Frozen section : Positive sentinal lymph nodes ( 5/5 ) and free margins .

Diagnosis:

Verified by :
Rana fathy



غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / جيهان حسين ابراهيم خليل | Request No. | : 13008872 |
| Patient No. | : 27511192301382 | Request Date | : 05/12/2023 |
| Path No. | : 4324/23 | Verification Date | : 25/12/2023 |
| Sex | : Female | Financial Category | : اورام دار السلام - اقتصادى |
| Age | : 48 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : .marwa mohamed youss |

## Pathology Report

Right breast conservative surgery , sentinal lymph nodes and axillary clearance , Up-front:
Invasive duct carcinoma , grade II with DCIS .
- Free margins .
- Stage: pT1c, pN3

Prof. Dr. Gamal Abdelaty
Dr. Marwa Mahmoud

Verified by :
Rana fathy



غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي للترانشونال – مصر معمل الباثولوجي
جوستاف روسي

| | | | |
|---|---|---|---|
| **Patient Name** | جيهان حسين ابراهيم | **Case No.** | **344848** |
| **Sex/Age** | **Female 50 Y** | **Patient No.** | **25323** |
| **Registered Date** | **29/7/2026 12:44:19 PM** | **Referred By Dr.** | **Interventional Radiology** |
| **Validation Date** | **22/8/2026 02:32:11 PM** | **Branch** | **Gustave Roussy** |
| **Reporting Date** | **22/8/2026 03:19:28 PM** | **Pathology No.** | **C-167/26** |

# Cytology Report

**Clinical Data:** Right sided pleural effusion in a patient with history of right MRM.

**Nature of specimen:** Right pleural effusion, Cytology, collected on 29/07/2026.

**Gross examination:**
Four alcohol fixed (H&E) stained smears & one cell block prepared from 20 cc of yellow fluid.

**Technique:**
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
SOX10 clone (SP267) & GATA3 clone (L50-823).

**Microscopic examination:**
Cytological examination of smears & cell- block prepared reveals cellular smears entangling malignant cells arranged in small cohesive clusters & papillary fronds.

**Results:**
*** IPT (All controls showed appropriate reactivity):**
- SOX10: Negative staining in tumor cells.
- GATA 3: Negative staining in tumor cells

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



CYTOLOGY
[[DOCTYPE:OTHER_CYTOLOGY]]

GUSTAVE ROUSSY
Gustave Roussy International Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

| | | | |
|---|---|---|---|
| Patient Name | جيهان حسين ابراهيم | Case No. | 344848 |
| Sex/Age | Female 50 Y | Patient No. | 25323 |
| Registered Date | 29/7/2026 12:44:19 PM | Referred By Dr. | Interventional Radiology |
| Validation Date | 22/8/2026 02:32:11 PM | Branch | Gustave Roussy |
| Reporting Date | 22/8/2026 03:19:28 PM | Pathology No. | C-167/26 |

**Diagnosis:**

**Right sided pleural effusion, Cytology:**

**- Positive for malignant cells.**

**- Further immunohistochemical studies for (MOC31, Calretinin & CK) are required to verify their exact origin.**

**Signature**

**Prof. Dr. Maha Guimei.**

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



CYTOLOGY
[[DOCTYPE:OTHER_CYTOLOGY]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Gihan Hussein Ibrahim Khalil
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 15-08-2026 07:01
Medical No. : 439467

NILE DIAGNOSTIC CENTER

## Tc-99m MDP SKELETAL SCINTIGRAPHY

**History:**
51-year-old female patient with history of surgically treated right breast cancer, received chemo-radiotherapy and hormonal treatment, referred for skeletal survey.

**Technique:**
Anterior and posterior whole-body scans 2 hours post injection of 20 mCi of the tracer.

**Findings :**

- Considering the last bone scan report dated 21/12/2024, the current scan revealed newly developed multiple, randomly distributed foci of variable tracer uptake keeping with bone secondaries. They are located at the sternum, around both shoulders, proximal parts of both humeri, dorso-lumbar vertebrae, bilateral scattered ribs, pelvic bones, proximal and distal parts of both femora as well as proximal parts of both tibiae.
- Osteoarthritic features are noted around big joints.

**IMPRESSION:**

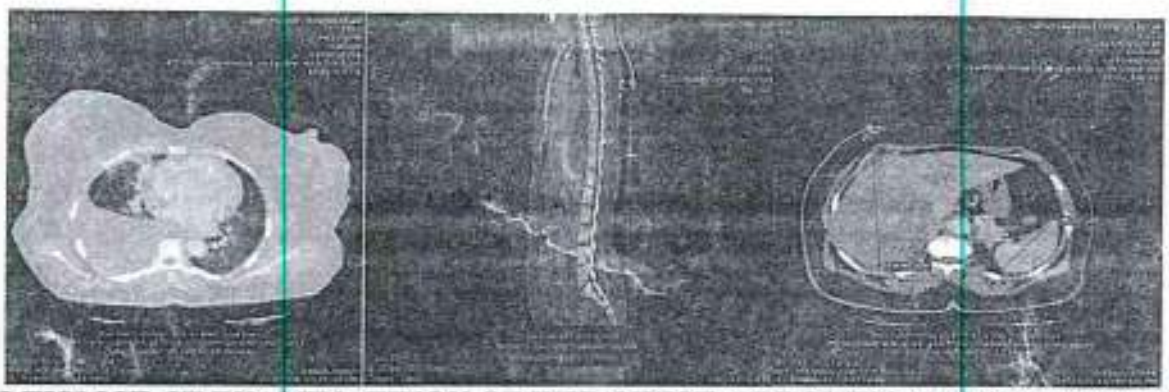
- Multiple active osseous lesions at the aforementioned sites most likely metastatic.
- Follow up bone scan is advisable to monitor response to therapy.

Much obliged
Dr. Alaa Nagy, MD
Nuclear medicine



Bone Scan
[[DOCTYPE:BONE_SCAN]]

| | |
|---|---|
| Patient's Name: | Gihan Hussein Ibrahim Khalil |
| Date: | 2026-07-14 |
| Referred by: | Prof.Dr. no doctor |
| Hospital: | Goustave Roussey International Egypt |



**MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-**

- Evident right MRM with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed. The overlying skin is till diffusely thickened,reaching to about 10mm in the peri-areolar region and few retro-pectineal dense lymph nodes are also still seen with the largest one measures about 12mm.
- Normal left breast.
- Newly-developed multiple variable sized left axillary lymph nodes are seen with loss of hilar fat contents in some of them with the largest one measures about 16mm.
- Marked right sided pleural effusion with retraction of the underlying lung towards the hilum.
- Newly-developed ground-glass densities are seen in the anterior segment of right upper pulmonary lobe and a patchy area of consolidation seen in the middle lobe and in the anterior segment of right lower pulmonary lobe.
- No sizable abnormally-enhancing pulmonary nodules ,other patchy areas of consolidations or calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No left pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Newly-developed foci of osteosclerotic bony lesions are seen widely scattering in the scanned skeleton.
- Average sized liver,measuring about 14.5 cm, showing a non-homogenous parenchymal attenuation and enhancement pattern with evidences of patchy low density seen in its right lobe.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 8.8 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

CT Scan
[[DOCTYPE:CT_SCAN]]

| | |
|---|---|
| **Patient's Name:** | **Gihan Hussein Ibrahim Khalil** |
| **Date:** | **2026-07-14** |
| **Referred by:** | **Prof.Dr. no doctor** |
| **Hospital:** | **Goustave Roussey International Egypt** |

**IMPRESSIONS:**

- **Evident right MRM with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed. The overlying skin is till diffusely thickened, and few right retro-pectineal dense lymph nodes are also still seen.**
- **Newly-developed multiple variable sized left axillary lymphadenopathy.**
- **Marked right sided pleural effusion with retraction of the underlying lung towards the hilum, denoting a progressive course.**
- **Newly-developed ground-glass densities in the anterior segment of right upper pulmonary lobe and a patchy area of consolidation also seen in the middle lobe and in the anterior segment of right lower pulmonary lobe.**
- **Newly developed foci of metastatic osteosclerotic bony lesions widely scattering in the scanned skeleton.**

**Prof/Hanan El-Ahwal.**

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

***Name Jihan ibraheem Hussein khaleel***

## Chest X-Ray Revealed

**Lung fields :** obliteration of the right cpa likely pleural effusion

Enlarged hilar bronchovascular markings and Mildly enlarged Cardio- mediastinal silhouette

**Diaphragm** : hemidiaphragms appear normal in contour

**Bones and soft tissue** : no acute osseus abnormality identified

Ultrasound
[[DOCTYPE:ULTRASOUND]]

GUSTAVE ROUSSY
INTERNATIONAL

١٢٨٤

جوستاف روسي انترناشونال

# ECHO REPORT

NAME: حيدر حسين ابراهيم

DATE: 23/8/2026

ID NUMBER: /

SEX: AGE:

| | | | | | |
|---|---|---|---|---|---|
| EF | 62% | 55-75% | PWT | 0.8 | 0.6-1.1cm |
| LVEDD | 4.3 | 3.5-5.8cm | AO.R | 2.3 | 3-3.7cm |
| LVESD | 2.7 | 2-4cm | LA | 3.6 | 2-4cm |
| FS | 28% | 25-45% | RV | | 0.7-2.3com(supine) |
| SWT | 0.9 | 0.6-1.1cm | AO.CUSP | | 1.5-2.6cm |

LV Dimensions Normal
LV Wall Thickness Normal
LV Diastolic Function GE
SWMA No. RWMA
LA and Ao Root Dimensions Normal
Mitral Valve Mild MR
Aortic Valve N
Tricuspid Valve trivial TR
Pulmonary Valve
Right Side and Systolic Function TAPSE: 2.1
Intracardiac Masses and Effusion

DR.

Cairo University
National Cancer Institute
Department of Diagnostic and
Interventional Radiology

جامعة القاهرة
المعهد القومي للأورام
قسم الأشعة التشخيصية والتداخلية

*Exam Information:*

| | |
|---|---|
| *Patient Name:* GEHAN Hussain | *Date of birth:* 19/11/1975 |
| *Arabic Name:* جيهان حسين | *Patient Gender:* F |
| *Patient RIS ID:* 336652<br>*Patient HIS ID:* 123018845 | *Exam Date:* 24/10/2023 |
| Written by:<br>Dr. Hoda Fathy | Approved by:<br>Dr. Hoda Fathy |

## *Nuclear Medicine Unit* وحدة الطب النووى

# *$^{99m}$Tc-MDP BONE SCAN*

Brief history: 48 years old female patient diagnosed with managed breast cancer.

Desired Information: - skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi $^{99m}$ Tc-MDP in the following manner:

Report:

- *No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.*
- *Spondylo-degenerative changes are seen over dorso-lumbar vertebrae.*
- *Pronounced osteoarthrosis noted over both knee joints.*
- *Both kidneys are normally visualized.*

## Conclusion:

***Negative study for the detection of any definite metastatic osseous deposits.***

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

| | | | |
|---|---|---|---|
| **Patient Name** | جيهان حسين ابراهيم | **Case No.** | **358888** |
| **Sex/Age** | **Female 50 Y** | **Patient No.** | **26042** |
| **Registered Date** | **18/8/2026 12:21:05 PM** | **Referred By Dr.** | **Out Patient** |
| **Validation Date** | **13/9/2026 02:35:20 PM** | **Branch** | **Gustave Roussy** |
| **Reporting Date** | **13/9/2026 02:58:25 PM** | **Pathology No.** | **SR-963/26** |

# Pathology Report

**Clinical Data:** Known Case Of Breast Cancer Presented With Right Upper Abdominal Skin Nodule.

**Nature of Specimen**: Block Revision.

**Gross Examination:**
- One paraffin block labelled by (716/26) was received.

**Technique:**
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- GATA-3, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

**Microscopic examination:**
- Histopathological examination of sections prepared revealed skin biopsy with dermal deposits of malignant cells disposed in clusters and individual cells of cohesive neoplasm with enlarged hyperchromatic nuclei.

**Results:**
*** IPT (All controls showed appropriate reactivity):**
- **GATA-3:** Positive nuclear reaction in tumor cells.
- **ER:** Weak nuclear reaction in 10% of tumor cells (score 3/8).
- **PR:** Moderate nuclear reaction in 5% of tumor cells (score 4/8).
- **Her2:** Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- **Ki67%:** 80%.

**Conclusion:**
**Known Case Of Breast Cancer With Abdominal Skin Nodule; Punch Biopsy Block Revision:**
**- Skin Dermal Deposits Of Breast Origin.**
**IPT:**
**- ER: Positive (score 3/8).**
**- PR: Positive (score 4/8).**
**- Her2: Positive (score 3+).**
**- Ki67%: 80%.**

****Scoring system: "ER, PR"***

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

| | | | |
|---|---|---|---|
| **Patient Name** | جيهان حسين ابراهيم | **Case No.** | **358888** |
| **Sex/Age** | **Female 50 Y** | **Patient No.** | **26042** |
| **Registered Date** | **18/8/2026 12:21:05 PM** | **Referred By Dr.** | **Out Patient** |
| **Validation Date** | **13/9/2026 02:35:20 PM** | **Branch** | **Gustave Roussy** |
| **Reporting Date** | **13/9/2026 02:58:25 PM** | **Pathology No.** | **SR-963/26** |

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

***** Scoring system: "Her2"***

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

**SIGNATURE**

***Dr. Walaa Ibrahim Ghonaim. MD.***

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

| | | | |
|---|---|---|---|
| **Patient Name** | جيهان حسين ابراهيم | **Case No.** | **358888** |
| **Sex/Age** | **Female 50 Y** | **Patient No.** | **26042** |
| **Registered Date** | **18/8/2026 12:21:05 PM** | **Referred By Dr.** | **Out Patient** |
| **Validation Date** | **13/9/2026 02:35:20 PM** | **Branch** | **Gustave Roussy** |
| **Reporting Date** | **24/9/2026 01:31:47 PM** | **Pathology No.** | **SR-963/26** |

# Pathology Report

**Clinical Data:** Known Case Of Breast Cancer Presented With Right Upper Abdominal Skin Nodule.

**Nature of Specimen**: Block Revision.

**Gross Examination:**
- One paraffin block labelled by (716/26) was received.

**Technique:**
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- GATA-3, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

**Microscopic examination:**
- Histopathological examination of sections prepared revealed skin biopsy with dermal deposits of malignant cells disposed in clusters and individual cells of cohesive neoplasm with enlarged hyperchromatic nuclei.

**Results:**
*** IPT (All controls showed appropriate reactivity):**
- **GATA-3:** Positive nuclear reaction in tumor cells.
- **ER:** Weak nuclear reaction in 10% of tumor cells (score 3/8).
- **PR:** Moderate nuclear reaction in 5% of tumor cells (score 4/8).
- **Her2:** Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- **Ki67%:** 80%.

**Conclusion:**
**Known Case Of Breast Cancer With Abdominal Skin Nodule; Punch Biopsy Block Revision:**
**- Skin Dermal Deposits Of Breast Origin.**
**IPT:**
**- ER: Positive (score 3/8).**
**- PR: Positive (score 4/8).**
**- Her2: Positive (score 3+).**
**- Ki67%: 80%.**

****Scoring system: "ER, PR"***

| | | | |
|---|---|---|---|
| **Patient Name** | جيهان حسين ابراهيم | **Case No.** | **358888** |
| **Sex/Age** | **Female 50 Y** | **Patient No.** | **26042** |
| **Registered Date** | **18/8/2026 12:21:05 PM** | **Referred By Dr.** | **Out Patient** |
| **Validation Date** | **13/9/2026 02:35:20 PM** | **Branch** | **Gustave Roussy** |
| **Reporting Date** | **24/9/2026 01:31:47 PM** | **Pathology No.** | **SR-963/26** |

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, > 1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

***** Scoring system: "Her2"***

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

**SIGNATURE**

***Dr. Walaa Ibrahim Ghonaim. MD.***